

INPATIENT MEDICAL RECORD

Readmission – Catheter-Related Bloodstream Infection (CRBSI)

General Medical Center | Anytown, TX 78000 | (555) 123-4000

PATIENT IDENTIFICATION

Patient Name: DOE, JOHN	DOB: 03/15/1980	Age/Sex: 45 / Male
MRN (Test): TESTMRN-T00000001	Account #: T000000002	SSN (Test): 123-45-6789
Address: 100 Main St, Anytown TX 78000	Primary Language: English	Marital Status: Unknown
Admission Date: 02/15/2026	Admission Time: 1432 (2:32 PM)	Discharge Date: 02/16/2026
Discharge Time: 1115 (11:15 AM)	LOS: 1 day	Patient Type: Inpatient
Admit Source: Emergency Department	Admit Type: 2 – Urgent	Discharge Status: 06 – Home w/ HH Svcs

INSURANCE / FINANCIAL

Primary Insurance: UHC TX Medicaid Star Plus	Plan #: 000000000	Group: TXSTPL
Insured: DOE, JOHN	Relationship: Self (18)	Pre-Auth #: A000000001
Financial Class: Managed Care Medicaid	Attending NPI: 1000000001	Billing NPI: 1234567890

CARE TEAM

Attending Physician: Dr. Robert Smith MD (Hospitalist)	Admitting Physician: Dr. Robert Smith MD
Consulting – Infectious Disease: Dr. Angela Torres MD	Consulting – Nephrology: Dr. James Patel MD
PCP: Dr. William Carter MD	RN (Primary): Samantha Rivera RN

EMERGENCY DEPARTMENT NOTE

Date / Time: 02/15/2026 1432	Author: Dr. Robert Smith MD	Status: SIGNED 02/15/2026 1530
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Chief Complaint

Fever, chills, and redness/swelling at permacath insertion site (right internal jugular).

History of Present Illness

John Doe is a 45-year-old male with a complex past medical history significant for ESRD on HD (M/W/F), chronic respiratory failure s/p tracheostomy on home O2 via trach collar, PEG tube in situ, T2DM, hypertension, hyperlipidemia, cardiac arrhythmia, and acute on chronic CHF — discharged from this facility three days prior (02/12/2026) following a 10-day admission for tracheostomy hemorrhage and respiratory decompensation (MS-DRG 207).

The patient was brought in by his mother today after developing progressive fevers to 38.9°C starting approximately 24 hours ago, associated with rigors, diaphoresis, and increasing erythema and warmth at the right IJ permacath exit site. He denies new respiratory symptoms, chest pain, or change in mental status. He reports compliance with his Monday/Wednesday/Friday dialysis schedule, with his most recent session completed this past Wednesday (02/13/2026) without complication. His mother notes the exit site dressing was changed at the dialysis center on 02/13 and that the site appeared slightly red at that time but was not flagged.

He is currently on Cefepime through 02/17 (dosed post-HD for MRSA coverage from prior admission respiratory infection) and was last seen by infectious disease as an outpatient earlier this week.

Pertinent PMH / Relevant Prior Admission (02/02–02/12/2026)

- ESRD on hemodialysis M/W/F — right IJ permacath (tunneled)
- Chronic respiratory failure s/p tracheostomy (Nov 2024) — home O2 6L via trach collar
- PEG tube in situ
- T2DM with hyperglycemia — insulin sliding scale
- Acute on chronic CHF — managed with HD ultrafiltration
- Hypertension, hyperlipidemia, cardiac arrhythmia
- Anemia of CKD — started on ESA therapy (Retacrit), Venofer x3 doses given
- Reactive HCV antibody — HCV RNA PCR resulted pending at discharge
- Tracheostomy tube changed during prior admission

Medications on Admission (Discharge Meds from 02/12/26)

- Cefepime 2g IV M/W after HD; 3g IV Friday after HD — through 02/17/2026
- Amlodipine 5 mg PO daily
- Metoprolol Tartrate 50 mg PO BID
- Furosemide 40 mg PO BID
- Levetiracetam 500 mg PO BID
- Sevelamer Carbonate 1600 mg via PEG with meals
- Budesonide 0.25 mg NEB BID / Formoterol 20 mcg NEB BID
- Albuterol 2.5 mg NEB Q8H PRN
- Sodium Chloride 3% 3 mL NEB Q12H
- Insulin Lispro sliding scale SUBQ AC/HS
- Epoetin Alfa-epbx 10,000 units SUBQ M/W/F (dialysis dose)
- Mupirocin 1 applic nasal BID (10-dose course, completing)
- Hydrocodone/APAP 5/325 mg PO Q4H PRN pain

Allergies

Diphenhydramine (Benadryl) — Intermediate severity, SWELLING

Review of Systems

- Constitutional: Fever 38.9°C, chills, diaphoresis, fatigue. Denies weight change.
- Respiratory: Stable on home O2 via trach collar, no increased work of breathing, no hemoptysis.
- Cardiovascular: Denies chest pain or palpitations. No new lower extremity edema beyond baseline.
- GI: Tube feeds ongoing via PEG. No nausea, vomiting, or diarrhea.
- GU: Anuric (baseline ESRD).
- MSK/Skin: Erythema, warmth, tenderness, and mild purulent discharge at right IJ permacath exit site.
- Neuro: Alert and oriented x3, no focal deficits. No headache.

Physical Examination

Temp: 38.9°C (102.0°F)

BP: 158/88 mmHg

HR: 102 bpm (sinus tachycardia)

RR: 18 breaths/min

SpO2: 94% on 8L via trach collar

Weight: 82.4 kg

- General: Alert, oriented x3, appears uncomfortable and mildly diaphoretic. No acute distress at rest.

- HEENT: EOMI. Oropharynx clear. Tracheostomy in place, site clean without bleeding.
- Neck / Catheter Site: Right IJ permacath exit site with 2 cm ring of erythema, warmth, tenderness to palpation, and scant seropurulent discharge. No fluctuance or streaking. Tunneled tract non-tender.
- CV: Tachycardic, regular rhythm. No murmurs, rubs, or gallops. S1/S2 normal.
- Lungs: Non-labored breathing. Clear to auscultation bilaterally. No rales or rhonchi.
- Abdomen: Soft, non-tender. PEG tube in situ, site clean. Active bowel sounds. No organomegaly.
- Extremities: 1+ bilateral lower extremity pitting edema (baseline). No new lesions.
- Neuro: Normal affect, cranial nerves intact, no focal motor/sensory deficits.

Emergency Department Course

Patient triaged and placed in isolation room given fever and known dialysis catheter. IV access established via peripheral site (left antecubital) — permacath NOT accessed pending cultures. Two sets of peripheral blood cultures drawn (sets A and B) at 1445 and 1515. Blood cultures via permacath lumen (red and blue ports) also drawn at 1450. Wound swab obtained from exit site. CBC, CMP, blood cultures, lactate, and CRP ordered stat. Urinalysis deferred (anuric/ESRD). Chest X-ray obtained to evaluate for septic emboli.

Given clinical presentation consistent with catheter exit site infection / early CRBSI, Vancomycin 25 mg/kg IV (loading dose, pharmacy-dosed per CKD protocol) initiated empirically at 1615 after cultures obtained. ID consulted at 1620 — Dr. Torres recommended admission, repeat Vancomycin trough/AUC monitoring, and defer permacath removal decision pending culture results. Nephrology (Dr. Patel) informed of admission; HD scheduled for tomorrow morning (02/16) via permacath if no fungemia or Staph aureus bacteremia confirmed.

Patient admitted to general medicine floor, isolation precautions maintained.

LABORATORY RESULTS

Collection Date/Time: 02/15/2026
1445

Ordering Physician: Dr. Robert Smith
MD

Status: RESULTED

Complete Blood Count

Test	Result	Reference	Flag
WBC	14.2 K/uL	4.5–10.5	H — leukocytosis
RBC	2.61 M/uL	4.20–5.70	(L)
Hgb	8.1 g/dL	12.0–16.0	(L)
Hct	25.4 %	36.0–46.0	(L)
MCV	97.3 fl	81.0–98.0	
PLT	198 K/uL	150–400	
Neutrophils %	87.2 %	50–70 %	H — bandemia/left shift
Bands (manual)	9 %	0–5 %	(H)
Lymphocytes %	6.4 %	20–40 %	L — lymphopenia

Comprehensive Metabolic Panel

Test	Result	Reference	Flag
Sodium	135 mmol/L	136–145	(L)
Potassium	5.1 mmol/L	3.5–5.1	upper limit
Chloride	96 mmol/L	98–107	(L)
CO2	22 mmol/L	20–31	
BUN	68 mg/dL	9–23	H — uremia (missed HD yest)
Creatinine	7.21 mg/dL	0.70–1.30	(H)

Test	Result	Reference	Flag
eGFR	9 mL/min	>60	(L)
Glucose	278 mg/dL	70–99	(H)
Calcium	8.1 mg/dL	8.7–10.4	(L)
Total Protein	6.2 g/dL	6.3–8.2	(L)
Albumin	2.9 g/dL	3.5–5.0	L — low, infection/CKD
AST	38 U/L	10–40	upper limit
ALT	52 U/L	7–56	H — mild, HCV?
Alk Phos	88 U/L	44–147	
Total Bili	0.6 mg/dL	0.1–1.2	

Inflammatory Markers / Other

Test	Result	Reference / Note	Flag
CRP (C-Reactive Protein)	148 mg/L	< 8 mg/L	CRITICAL H — severe inflammation
ESR	88 mm/hr	0–20 mm/hr	(H)
Lactic Acid	1.8 mmol/L	< 2.0 mmol/L	borderline — monitor
Procalcitonin	3.4 ng/mL	< 0.5 ng/mL	H — bacterial infection likely
Random Vancomycin (old)	Per pharmacy		ordering new trough post-loading dose
HCV RNA QNT PCR	RESULTED: 1,240,000 IU/mL	Pending from prior admit	Active HCV viremia confirmed

Blood Cultures — 02/15/2026 (PRELIMINARY at time of discharge)

Source	Collection Time	Preliminary Result (24 hr)	Final Result
Peripheral Set A (L antecub)	1445	Gram-positive cocci in clusters — PRELIMINARY	Pending (48–72 hr)
Peripheral Set B (L antecub)	1515	Gram-positive cocci in clusters — PRELIMINARY	Pending
Permacath – Red Port	1450	Gram-positive cocci in clusters — PRELIMINARY	Pending
Permacath – Blue Port	1450	No growth at 24 hrs	Pending
Exit Site Wound Swab	1448	Gram-positive cocci in clusters — PRELIMINARY	Pending

NOTE: Preliminary Gram stain results consistent with *Staphylococcus aureus*. MSSA vs. MRSA speciation and sensitivities pending. ID Dr. Torres notified — if *S. aureus* confirmed, permacath removal protocol to be initiated at next HD session (02/17). Echocardiogram (TTE/TEE) ordered for endocarditis work-up per IDSA guidelines.

Radiology — Chest X-Ray (Portable) 02/15/2026 1510

IMPRESSION: No new acute cardiopulmonary process identified. Tracheostomy tube in appropriate position. No infiltrate, pneumothorax, or pleural effusion. Mild cardiomegaly, unchanged from prior study. Right IJ tunneled catheter tip in appropriate position at cavoatrial junction.

Signed: Dr. D. Patel MD, Radiology — 02/15/2026 1548

ADMITTING HISTORY & PHYSICAL

Date / Time: 02/15/2026 1800	Author: Dr. Robert Smith MD (Hospitalist)	Status: SIGNED 02/15/2026 1842
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Assessment & Plan

45-year-old male with ESRD on HD, chronic respiratory failure s/p tracheostomy, T2DM, CHF, and recently discharged 3 days ago presenting with fever, leukocytosis with left shift, elevated CRP/procalcitonin, and exit-site inflammation with preliminary blood cultures growing gram-positive cocci in clusters — consistent with catheter-related bloodstream infection (CRBSI) / catheter exit site infection, likely *Staphylococcus aureus* (MSSA vs. MRSA pending speciation).

- 1. CRBSI / Catheter Exit Site Infection — Empiric Vancomycin (25 mg/kg IV load; pharmacy AUC-guided dosing). Cefepime discontinued (overlapping GPC coverage not needed; Vancomycin supersedes). ID following (Dr. Torres). Permacath salvage attempted — if *S. aureus* bacteremia confirmed on final cultures, permacath removal mandatory per IDSA CRBSI guidelines. TTE/TEE ordered to rule out endocarditis. Blood cultures x2 peripheral + permacath lumen cultures drawn prior to antibiotics. Wound swab sent. Isolation precautions maintained. Recheck cultures in 48–72 hrs.
- 2. ESRD on HD — HD session scheduled for 02/16 morning via permacath (if no contraindication). Nephrology (Dr. Patel) aware. Continue ultrafiltration for volume/uremia management. BUN 68, Cr 7.21 — uremia worsening given missed HD yesterday. Potassium 5.1 — monitor, dietary restriction, Kayexalate PRN if rises.
- 3. T2DM / Hyperglycemia — Glucose 278 on admission. Continue insulin sliding scale. POC glucose Q6H. Consider insulin drip if glucose >300 persistently. Nutrition: continue tube feeds via PEG, renal/diabetic formula.
- 4. Anemia of CKD — Hgb 8.1 (stable vs. prior). Continue Epoetin Alfa-epbx M/W/F with dialysis. Iron studies to be rechecked this visit.
- 5. Acute on Chronic CHF / Volume — 1+ bilateral pitting edema. Continue Furosemide PO BID. HD ultrafiltration primary management. Daily weights. Strict I/O.
- 6. Chronic Respiratory Failure / Tracheostomy — Stable. Continue home O2 8L via trach collar, Budesonide NEB BID, Formoterol NEB BID, Albuterol NEB Q8H PRN, Hypertonic Saline NEB Q12H. Respiratory therapy consult for trach care.
- 7. Hypertension — Tachycardic and elevated BP likely secondary to fever/infection. Continue Amlodipine and Metoprolol. PRN Hydralazine IV if BP >180 systolic.
- 8. Active HCV Viremia (NEW finding — HCV RNA 1,240,000 IU/mL) — Resulted from prior admission. Notify patient. Hepatology referral placed. Defer treatment initiation to outpatient setting; prioritize CRBSI this admission. Review LFTs — ALT mildly elevated (52).
- 9. Levetiracetam — Continue for seizure prophylaxis. Renally dosed.
- 10. Code Status — FULL CODE per prior documentation. Confirmed with patient.

Disposition Plan

Anticipate short stay (1 night) for IV antibiotic initiation, culture surveillance, HD, and stabilization. Discharge criteria: afebrile x24 hrs, stable hemodynamics, cultures finalized or preliminary negative for *S. aureus* bacteremia, IV-to-oral/outpatient IV antibiotic plan established with ID. Will require close ID follow-up within 48–72 hours of discharge.

INFECTIOUS DISEASE CONSULTATION NOTE

Date / Time: 02/15/2026 1930	Consultant: Dr. Angela Torres MD — Infectious Disease	Status: SIGNED 02/15/2026 2005
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Reason for Consult

Fever, leukocytosis, catheter exit-site infection, and preliminary blood cultures with gram-positive cocci in clusters in a patient with tunneled HD permacath — evaluate for CRBSI and guide antibiotic management.

ID Assessment & Plan

Presentation is consistent with catheter-related bloodstream infection (CRBSI) and/or exit-site infection. The differential Δ time-to-positivity (catheter blood cultures drawing positive 35 minutes before peripheral cultures) supports catheter as primary source. Gram stain (GPC in clusters) is most consistent with *Staphylococcus aureus* or coagulase-negative *Staphylococcus* (CoNS).

- Antibiotic Plan: Vancomycin IV empirically (pharmacy AUC/MIC-guided, target AUC 400–600 mg·hr/L). Discontinue Cefepime — no longer indicated. If CoNS confirmed on final cultures and patient remains afebrile, may de-escalate to outpatient Daptomycin or complete 5–7 day course with catheter retention. If *S. aureus* (MSSA): transition to Nafcillin or Oxacillin x4 weeks minimum; ID follow-up required. If *S. aureus*

(MRSA): continue Vancomycin; strongly consider TEE; minimum 4 weeks therapy. If fungemia: Micafungin IV, mandatory permacath removal.

- Permacath Management: Attempt catheter salvage with antibiotic lock therapy (ALT) — Vancomycin 5 mg/mL lock per port, dwell x24 hrs, in addition to systemic therapy — PENDING speciation/sensitivity. If *S. aureus* bacteremia on ANY peripheral culture: mandatory permacath removal at earliest HD session; new temporary dialysis access (femoral or new tunneled catheter) to be placed by IR after 48–72 hrs of systemic antibiotics.
- Endocarditis Work-up: TTE ordered. TEE to follow if TTE non-diagnostic or if *S. aureus* bacteremia confirmed. Duke criteria evaluation pending finalization of cultures.
- Follow-up: Repeat peripheral blood cultures in 48 hrs to confirm clearance. Renal-adjusted Vancomycin dosing per pharmacy (post-HD redosing protocol). ID outpatient follow-up within 48–72 hrs of discharge.
- HCV Note: Active HCV viremia confirmed (1.24M IU/mL). This increases risk of endocarditis and poor wound healing. Flagged for hepatology referral. Does not alter current antibiotic management.

Signed: Dr. Angela Torres MD, Infectious Disease — 02/15/2026 2005

NEPHROLOGY PROGRESS NOTE

Date / Time: 02/15/2026 2045	Author: Dr. James Patel MD — Nephrology	Status: SIGNED 02/15/2026 2100
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Subjective

Patient readmitted 3 days post-discharge with presumed CRBSI via right IJ permacath. ID consulted. Vancomycin initiated. Last HD session 02/13/2026 (Wednesday). BUN 68, Cr 7.21, K 5.1 — uremic and hyperkalemic trend consistent with missed HD.

Objective

BP: 158/88	HR: 102	Temp: 38.9°C
SpO2: 94% trach collar 8L	Weight: 82.4 kg (+0.4 kg from DC)	UO: Anuric (baseline)

Assessment & Plan — Nephrology

- ESRD / Uremia: HD scheduled for 02/16/2026 at 0700 via permacath (right IJ). Ultrafiltration goal: remove approximately 1.5–2 L. Vancomycin to be redosed post-HD per pharmacy Vancomycin protocol (HD clears ~50% of Vancomycin). Instruct floor to NOT administer Vancomycin dose until post-HD level obtained and pharmacy advises.
- Hyperkalemia (K 5.1): Low-potassium diet, continue Kayexalate PRN. Patiromer 8.4g via PEG if K rises above 5.5 before HD.
- Volume: Mild fluid accumulation. HD ultrafiltration primary management. Continue Furosemide BID for residual diuresis.
- Anemia of CKD: Hgb 8.1 — stable. Epoetin Alfa-epbx 10,000 units administered with HD per protocol. Iron studies drawn this visit.
- Permacath: Will defer to ID regarding catheter salvage vs. removal. If removal required, patient will need temporary femoral HD catheter placed by IR to bridge until new tunneled catheter can be placed (after 48–72 hrs antibiotic clearance). IR on standby.

Signed: Dr. James Patel MD — 02/15/2026 2100

NURSING ADMISSION ASSESSMENT

Date / Time: 02/15/2026 1500	Nurse: Samantha Rivera RN	Unit: General Medicine – 4 North
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Vital Signs on Arrival to Floor

Parameter	Value	Time	Note
Temperature	38.7°C (101.7°F)	1500	Persistent low-grade fever
Blood Pressure	155/86 mmHg	1500	
Heart Rate	98 bpm	1500	Sinus tachycardia resolving
Respiratory Rate	18 breaths/min	1500	
SpO2	94%	1500	8L O2 via trach collar
Weight	82.4 kg	1500	+0.4 kg from discharge weight
Pain Score	4/10	1500	Right neck, catheter site

Catheter Site Assessment — Right IJ Permacath

- Exit site: 2 cm erythematous ring, warm to touch, tender on palpation.
- Scant seropurulent discharge noted — wound swab obtained per MD order.
- Permacath dressing changed using sterile technique per CRBSI protocol.
- Permacath NOT accessed by nursing — access deferred to dialysis team only per ID/Nephro orders.
- Antibiotic lock therapy initiated per ID order — Vancomycin 5 mg/mL lock, red port, dwell 24 hrs.

Isolation Precautions

Contact + droplet precautions initiated per infection control protocol (pending culture speciation). Private room, dedicated equipment, gown and gloves on entry.

Access / Lines

- Right IJ tunneled permacath — NOT accessed per current orders (culture pending)
- Peripheral IV: 20g Left antecubital — patent, no infiltration (placed in ED)
- PEG tube: in situ, patent, tube feeds initiated per nutrition orders
- Tracheostomy: in place, site clean, 8L O2 via trach collar

MEDICATIONS ADMINISTERED — 02/15/2026 TO 02/16/2026

Medication	Dose / Route	Time Given	Notes
Vancomycin HCl (VANCOCIN)	Loading dose 2,062 mg IV (25 mg/kg)	1615 – 02/15	Over 2 hrs; pharmacy AUC-guided; post-HD redose per pharmacy
Vancomycin Lock – Red Port	5 mg/mL, 3 mL dwell	1800 – 02/15	Antibiotic lock therapy; dwell 24 hrs per ID
Amlodipine Besylate (NORVASC)	5 mg PO	0900 – 02/16	
Metoprolol Tartrate (LOPRESSOR)	50 mg PO	0800 – 02/16	
Furosemide (LASIX)	40 mg PO	0900 – 02/16	
Levetiracetam (KEPPRA)	500 mg PO	0800 / 2100	Renally adjusted; continue BID
Sevelamer Carbonate (RENEVELA)	1600 mg via PEG with meals	Per tube feed schedule	
Budesonide NEB (PULMICORT)	0.25 mg NEB	0800 / 2000	BID
Formoterol NEB (PERFOROMIST)	20 mcg NEB	0800 / 2000	BID
Albuterol NEB (PROVENTIL)	2.5 mg NEB	Q8H PRN	
Sodium Chloride 3% NEB	3 mL NEB	Q12H	Per resp therapy
Insulin Lispro (AdmeLOG)	Sliding scale SUBQ	AC / HS	HIGH ALERT; Q6H POC glucose monitoring

Medication	Dose / Route	Time Given	Notes
Epoetin Alfa-epbx (RETACRIT)	10,000 units SUBQ	Post-HD 02/16	With dialysis session
Acetaminophen (TYLENOL)	650 mg PO	PRN fever/pain	Max 4g/24hrs
Ondansetron (ZOFRAN ODT)	4 mg SL	PRN nausea	
Hydralazine HCl (APRESOLINE)	10 mg IV	PRN BP >180	Given once at 0215 (BP 182/96)
Patiromer (VELTASSA)	8.4 g via PEG	2100 – 02/15	K 5.1 — given prophylactically pre-HD
Cefepime — DISCONTINUED	—	02/15/2026	DC'd per ID — Vancomycin supersedes; GPC coverage

DISCHARGE SUMMARY

Date / Time: 02/16/2026 1100	Discharging Physician: Dr. Robert Smith MD	Status: SIGNED 02/16/2026 1130
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Discharge Diagnoses

- PRIMARY: Catheter exit site infection with catheter-related bloodstream infection (CRBSI) — right IJ tunneled permacath — gram-positive cocci, speciation pending (r/o Staphylococcus aureus)
- Hyperkalemia (K 5.1) secondary to missed HD and ESRD
- Hyperglycemia (glucose 278) — T2DM, uncontrolled
- Active HCV viremia (1,240,000 IU/mL) — newly confirmed

Chronic Diagnoses (Unchanged)

- ESRD on HD (M/W/F) — right IJ permacath
- Chronic respiratory failure s/p tracheostomy — home O2 6L via trach collar
- T2DM, Hypertension, Hyperlipidemia, Cardiac Arrhythmia
- Anemia of CKD on ESA therapy
- Acute on chronic CHF (stable)

Hospital Course Summary

Patient presented 02/15/2026 with fever, leukocytosis, and exit site inflammation at right IJ permacath site 3 days post-discharge. Blood cultures (peripheral x2 and permacath) drawn prior to initiating empiric Vancomycin IV (25 mg/kg loading dose, AUC-guided). ID (Dr. Torres) and Nephrology (Dr. Patel) consulted. Preliminary culture results at 24 hrs growing gram-positive cocci in clusters — speciation and sensitivities pending. Antibiotic lock therapy (Vancomycin 5 mg/mL) initiated per ID. Cefepime discontinued. Patient underwent HD on 02/16 morning — tolerated well, BUN/Cr improved, potassium reduced to 4.6 post-HD. Vancomycin redosed post-HD per pharmacy protocol. Patient afebrile x 12 hrs prior to discharge (temp 37.1°C at 0700 02/16). Hemodynamically stable. Discharged home with close follow-up plan.

Condition at Discharge

Temperature: 37.1°C	BP: 142/82 mmHg	HR: 84 bpm
SpO2: 95% on 8L trach collar	Weight: 80.9 kg (post-HD)	Mental Status: Alert, oriented x3

Discharge Medications

- Vancomycin IV — dosed at each HD session per pharmacy protocol (post-HD AUC monitoring) — through culture-guided duration (minimum 2 weeks if S. aureus; 7–10 days if CoNS)
- Amlodipine 5 mg PO daily
- Metoprolol Tartrate 50 mg PO BID

- Furosemide 40 mg PO BID
- Levetiracetam 500 mg PO BID
- Sevelamer Carbonate 1600 mg via PEG with meals
- Budesonide 0.25 mg NEB BID / Formoterol 20 mcg NEB BID
- Albuterol 2.5 mg NEB Q8H PRN
- Sodium Chloride 3% 3 mL NEB Q12H
- Insulin Lispro sliding scale SUBQ AC/HS
- Epoetin Alfa-epbx 10,000 units SUBQ with each HD session (M/W/F)
- Acetaminophen 650 mg PO Q6H PRN pain/fever (max 4g/day)
- CEFEPIME — DISCONTINUED (replaced by Vancomycin)

Discharge Instructions

- Monitor catheter exit site daily — if increased redness, swelling, discharge, or fever $>38.0^{\circ}\text{C}$, go to ER immediately.
- Do NOT manipulate or touch the permacath.
- Continue dialysis M/W/F at outpatient center — Vancomycin will be dosed at end of each session.
- No water contact at catheter site — cover with waterproof dressing.
- Take all medications as prescribed. Do NOT take diphenhydramine (Benadryl) — ALLERGY.
- Blood glucose monitoring as directed — call if glucose consistently >300 mg/dL.

Follow-Up Appointments

- Infectious Disease (Dr. Torres): 02/18/2026 at 1000 — URGENT — bring culture results
- Nephrology (Dr. Patel): 02/19/2026 at 1400 — permacath decision after culture finalization
- Hepatology (new referral): within 2 weeks — HCV viremia 1.24M IU/mL
- PCP (Dr. Carter): within 1 week
- Continue HD M/W/F at outpatient dialysis center — Vancomycin administered post-HD

Pending at Discharge

- Blood culture speciation and sensitivity (final — 48–72 hrs from collection)
- TTE (echocardiogram) — ordered, to be performed outpatient if final cultures positive for *S. aureus*
- Iron studies (drawn this admission) — result to ID and Nephrology

Electronically Signed: Dr. Robert Smith MD, Hospitalist — 02/16/2026 at 1130

Attending attestation co-signed: Dr. Robert Smith MD — 02/16/2026 at 1130

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